

Headache: diagnosis in adults

NICE 2012, CG150, NICE 2015, NG12, NICE 2019, NG127



Red Whale

GEMS

Guidelines & Evidence Made Simple

Assess for headache red flags

Headache red flags can be remembered with the handy mnemonic HEAD CRISIS C:

- H** Head trauma (in the past 3m).
- E** Exacerbated by triggers: cough, Valsalva, sneezing, exercise, change in posture.
- A** Acute narrow-angle glaucoma/giant cell arteritis.
- D** Deficit (neurological), cognitive impairment or a change in personality.
- C** Change in character of the headache.
- R** Roasting hot (fever and worsening headache).
- I** Impaired consciousness.
- S** Sudden onset (if maximum intensity within 5 minutes, refer immediately to exclude subarachnoid haemorrhage).
- I** Immunocompromised.
- S** Sick (vomiting) without other obvious cause.
- C** Cancer: previous cancer known to metastasise to brain OR <20y and cancer history.

If **ANY** red flags present:
NICE says consider
investigating/referral
as appropriate

If **NEW** headache with
these features:
investigate/refer.

Headache alone is not a red flag symptom for a brain tumour. The NICE suspected cancer guidance recommends considering urgent direct-access MRI brain scan if progressive, subacute loss of CNS function.

No red flags: what could it be?

HISTORY crucial! Ask about headache symptoms, other symptoms and impact on activities.

Consider a headache diary for 8w that includes headache features (frequency, duration, severity), associated symptoms, medication used and efficacy, precipitants and any relation of headache to menstruation.

Would a scan help? Often considered for cluster headaches, rarely for other types in the absence of red flags.

	Headache symptoms	Associated symptoms
Cluster headache (refer to confirm diagnosis)	- Strictly unilateral. - Usually around/above the eye and along the side of the face/head. - Severe/very severe. - Lasts 15–180min. - Can get many episodes within a day.	<u>Ipsilateral</u> autonomic features (on the same side as the headache): - Red/watery eye. - Constricted pupil. - Swollen/drooping eyelid. - Nasal congestion/rhinorrhoea. - Facial sweating. Often restless or agitated.
Migraine	- Unilateral or bilateral. - Pulsating, throbbing or banging. - Moderate/severe. - Lasts 4–72h in adults (1–72h in those 12–17y).	- Sensitivity to light/sound. - Nausea/vomiting. - Aura. Aggravated by routine activities of daily living OR causes activity avoidance.
Tension headache	- Bilateral. - Pressing/tightening (non-pulsating). - Mild/moderate severity. - Lasts 30min–continuous.	No associated symptoms. Not aggravated by routine activities of daily living.
Medication-overuse headache	Location, severity and duration all variable!	Consider if frequent use of acute treatment for headache for ≥3m: - Paracetamol, aspirin or an NSAID on ≥15d/m. - Triptan, opioid, ergot or combination analgesia on ≥10d/m. Usually not aggravated by routine activities.

Migraines and tension headaches can be defined as episodic or chronic (precise definition in main article). This matters when it comes to CGRP drugs used for migraine in secondary care. **Chronic migraine and chronic tension headache often overlap. If any features of migraine, diagnose chronic migraine.**

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